Nursing Essentials

Priority: Grunting, flaring, retractions, apnea, or central cyanosis needs prompt assessment and respiratory support.

FOCUS 1

Support lungs

- **Typical setting:** prematurity and surfactant deficiency; symptoms begin early.
- **Support:** temperature control, monitoring, respiratory support, and surfactant when ordered.
- **Watch:** work of breathing, oxygen requirement, blood gases when ordered, and signs of fatigue.

FOCUS 2

Assess course

- **Mechanism:** delayed clearance of fetal lung fluid; often occurs in term or late-preterm infants.
- **Course:** commonly improves over 24–72 hours, but the team must evaluate other causes.
- **Feeding safety:** significant tachypnea may make oral feeding unsafe; follow the prescribed nutrition plan.

FOCUS 3

Oxygenate

- **Meconium aspiration:** can impair ventilation and oxygenation; respiratory support is individualized.
- **No routine suction:** meconium alone does not justify routine tracheal suctioning; effective ventilation takes priority.
- **PPHN:** impaired transition of pulmonary circulation can produce severe hypoxemia; specialized evaluation and treatment are required.

Escalation and special cautions

Make it stick: For apnea, gasping, or bradycardia, follow the neonatal resuscitation algorithm. Ventilation is the central early intervention.

Focus	What to know	Nursing application
Do not assume TTN	infection, pneumothorax, congenital heart disease, and other disorders may resemble it.	Reassess the diagnosis
Sudden worsening	assess the airway, equipment, breath sounds, perfusion, and need for emergency assistance.	Check airway and perfusion
Suspected diaphragmatic hernia	avoid routine bag-mask inflation; prepare for the specialized airway and gastric-decompression plan.	Use the specialized plan

RED FLAGS · ESCALATE PROMPTLY

Escalate apnea, central cyanosis, increasing oxygen need, exhaustion, sudden asymmetry of breath sounds, or cardiovascular deterioration.